

AI-Powered HMS Synthetic Clinical Dataset

Clinical Note

Patient Name	Ho My Xuan	MRN	MRN-0032
DOB	1995-05-14	Gender	Female
Encounter Date	2024-06-28	Department	Orthopedics
Attending Provider	BS. Le Hoang Phuc	Status	active

Synthetic Data Warning: This document is generated for software testing, OCR parsing, chunking, embedding, access-control validation, and Graph RAG demonstrations. It is not valid medical advice and must not be used for patient care.

Subjective

Ho My Xuan is a 31-year-old patient in Orthopedics presenting for follow-up of pain and impaired mobility. PMH includes fracture repair or degenerative joint disease, no acute neurovascular compromise. Patient reports symptoms are stable overall. Denies severe chest pain, syncope, active bleeding, or new focal neurologic deficit unless otherwise documented. Current meds include acetaminophen 500mg QID PRN; ibuprofen 400mg TID PRN; pantoprazole 40mg daily. Allergies: NKDA.

Objective

Vitals: BP 142/72 mmHg, HR 94 bpm, RR 14/min, Temp 36.8 C, SpO2 96% RA, weight 87 kg. Exam: alert, oriented, no acute distress. Cardiopulmonary exam without unstable findings. Abdomen soft, non-tender. Extremities show no acute deformity; edema varies by department-specific condition. Labs reviewed below; medication list reconciled.

Recent Lab Snapshot

Date	Analyte	Value	Unit	Reference Range	Status
2026-05-02	Creatinine	1.24	mg/dL	0.7-1.3	Normal
2026-05-02	eGFR	66.0	mL/min/1.73m2	>=60	Normal
2026-05-02	Glucose	115.0	mg/dL	70-99	High
2026-05-02	HbA1c	5.6	%	<5.7	Normal
2026-05-02	BNP	301.0	pg/mL	<100	High
2026-05-02	AST	32.0	U/L	10-40	Normal
2026-05-02	ALT	35.0	U/L	7-56	Normal
2026-05-02	Potassium	4.2	mmol/L	3.5-5.0	Normal
2026-05-02	Hemoglobin	13.6	g/dL	12.0-17.5	Normal

Assessment

Primary assessment: post-procedure musculoskeletal pain or osteoarthritis with mobility limitations. Patient is clinically stable for ongoing management. Risk considerations include renal dosing, medication safety, fall risk, and need for timely escalation if red-flag symptoms occur.

Plan

Assess pain score and mobility daily. Reinforce PT plan and fall precautions. Check wound or surgical site for erythema, drainage, or fever. Provide patient education, update care team via SBAR, and document any access-relevant clinical justification before AI-assisted retrieval of PHI.

Detailed Care Coordination Notes

Problem List

1. post-procedure musculoskeletal pain or osteoarthritis with mobility limitations. 2. Medication safety monitoring. 3. Functional status and discharge readiness. 4. Follow-up coordination with Orthopedics.

Safety Triggers

Escalate to responsible clinician for SBP < 90, HR > 130, SpO2 < 92%, acute mental status change, active bleeding, severe pain, fever with neutropenia risk, or rapidly rising Cr/K.

AI Retrieval Tags

access_tags: read, summary, labs, medication, cardiology as applicable. Suggested document_type for ingestion: clinical_note.